



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نجوى عبدالرحمن ابو عوف عبدالرحمن

العنوان : ++++++

المحافظة : القاهرة

رقم البطاقة : 26408210103349 رقم التليفون : 01118873807

العمر (تاريخ الميلاد) : 21-08-1964 المهنة : حاصل علي دبلوم

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي
المقر بما فيه
التوقيع: (-----)

التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني	الوظيفة: -----	التوقيع: -----
الاسم: د/ندى حسن	الوظيفة: -----	التوقيع: -----
الاسم: د/نرمين رمضان	الوظيفة: -----	التوقيع: -----

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
قسم : -----
جهة العلاج : -----
نفقات : -----
ملاحظات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: نجوى عبدالرحمن ابو عوف عبدالرحمن National ID: 26408210103349	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



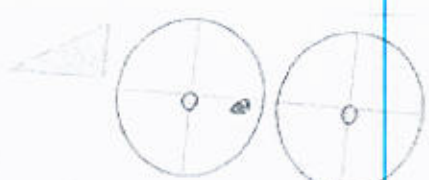


مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

مصرية
قوة
IT

MDT FORM

Name	د. محمد عبد الرحمن
Age	56 y
MRN	
ID	6778-ACI-1-2229
Co morbidity	HELVUSAEAN



Rr c T₂ N₀

(Small Breast)

Clinical Data

Bx:	SM:
IHC:	Mets Workup:
ER:	
PR:	
HerT:	
KiTV:	
T N M	

ID 4
(unilob)
A

COMMENT:

CESM → unifocal. + small
R axilla → NAD intraductal
exent.

MDT OUTCOME & PLAN

14/8/2024 → clips

- Noady. A.I / assess clinically / mri.
(6-8 mms)

Then Rt BES, SUNB

MEDICAL ONCOLOGY	SIGNATURE	RADIOLOGY	PATHOLOGY

Name: نجوى عبد الرحمن ابو عوف عبد الرحمن			
MR. : 6497			
Detailed Medical Report			
		السيد	
تحية طيبة وبعد,,			
		إيماء للطلب المقدم من	
بشأن تقرير طبي للمريض			
رقم طبي:		بقسم:	
نشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الخروج :		تاريخ وساعة الدخول :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
The patient is scheduled for laboratory workup prior to administration of zoledronic acid (Zometa); pending labs are to be obtained today. The patient will also continue anastrozole (Arimidex) therapy, with a two-month supply dispensed by the pharmacy. The patient is a 61-year-old individual with a positive family history of malignancy. Pathology revealed estrogen receptor (ER) and progesterone receptor (PR) scores of 8/8 and HER2 negativity, staged as clinical T2 N0. The patient completed neoadjuvant anthracycline-taxane (NAHT) chemotherapy for eight months, followed by breast-conserving surgery (BCS) and radiation therapy in March 2022. Pathology after surgery demonstrated ypT1c N1a disease. A port was placed, and hormonal therapy with an aromatase inhibitor was continued, with the most recent refill in June 2026. At the time of this note the patient is free of detectable malignancy, with residual duct ectasia in the left breast, and remains on anastrozole.			
تحريراً في ,			
التاريخ :		20-09-2026	
الوقت :		شريف الشناوى	
الطبيب المعالج :		يعتمد ,	



Name: Nagwa AbdelRahman	Date: 27-07-2021
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POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Right breast UIQ two interconnected irregular speculated mass lesions the largest measuring 1.6 cm in diameter.
- Right axillary level I and II pathologically enlarged lymph nodes the largest measures 4.5 x 2.5 cm in maximum axial dimensions.
- Few bilateral scattered tiny pulmonary nodular densities ranging from 3-4 mm.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. It shows few right hepatic lobe about two hepatic cystic focal lesions the largest is seen at segment V, it measures 1.5 X 1 cm.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- underfilling of the urinary bladder.
- Non visualized uterus ?? surgically removed.
- No abdominal or pelvic masses or collections.
- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.



National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة



Pathology Report

Name: نجوى عبدالرحمن ابو عوف عبدالرحمن

Age: 56 years.

Clinical data: Right breast.

Specimen type: Tru cut biopsy.

Date (Received): 13 /7/2021.

Date (reported): 15 /7/2021

Lab. Number: 1528 / 21

Hospital Number: ميادة

Gross Examination:

Five grayish white tissue cores, longest 2 cm. Totally submitted.

Microscopic examination:

Histopathological examination of sections prepared from the Tru cut specimen received reveals an infiltrating malignant epithelial growth. The tumor is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II]. No intraduct component is detected.

Diagnosis:

Right breast mass ; Tru cut biopsy:

Invasive duct carcinoma, Grade II.

For Further marker study (ER , PR , HER-2 & Ki 67)

Prof. Dr. Lubna Omar

Dr. Samah Sayed



National Women's Health Initiative
Pathology Lab
معمل باثولوجي المبادرة القومية لصحة المرأة

استأجر د. سامح السيد
مبادرة

Pathology Report

Name: نجوى عبدالرحمن ابو عوف عبدالرحمن Age: 56 years.
Clinical data: Right breast.
Specimen type: Tru cut biopsy.
Date (Received): 13 /7/2021. Date(reported): 29 /7/2021
Lab. Number: 1528 / 21 Hospital Number: مبادرة

Gross Examination:

Five grayish white tissue cores, longest 2 cm. Totally submitted.

Microscopic examination:

Histopathological examination of sections prepared from the Tru cut specimen received reveals an infiltrating malignant epithelial growth. The tumor is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II]. No intraduct component is detected .

Diagnosis:

Right breast mass ; Tru cut biopsy:

Invasive duct carcinoma, Grade II .

Biomarkers:

- ER: Strong nuclear reaction in 70% of tumor cells (Allred score 8/8)
- PR: Strong nuclear reaction in 70% of tumor cells (Allred score 8/8)
- Her-2: Faint incomplete membrane reaction in < 10% of tumor cells , negative (score 1)
- Ki 67: 10 % nuclear reaction

Prof. Dr. Lubna Omar

Dr. Samah Sayed



OPINION:

- *Right breast malignant looking mass lesion.*
- *Few bilateral scattered tiny pulmonary nodular densities likely non-specific.*
- *Right hepatic lobe few scattered cystic lesions for US assessment.*
- *No definite osseous metastatic deposits.*

Dr. Mohammed Mohsen, MSc.

Dr. M. Gomaa, M.D.

Patient Name: Nagwa Abd El Rahman Abo`Ouf Abd El Rahman	Date : 25/10/2023
Exam : MAMMOGRAPHY BOTH	Age : 59 Year(s)
Medical ID: 2932/2021	Pat. ID: 21050

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indication:

- Right breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

Right breast:

- Operative bed para-areolar scarring with related notable parenchymal distortion and related heterogenous palpable area at 2-3 o'clock, measuring about 17 x 12 mm, possibly fat necrosis.
- No suspicious micro calcifications
- No sizable collections.
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

- Scattered fibro glandular density (ACR B)
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing operative bed changes along with (2-3 o'clock) focal area of possible fat necrosis (BIRADS 3), for follow up after 6 months.
- Normal study of the left breast (BIRADS 1)

Signature Dr. Peter Nashat

25/10/2023 1:17 PM

Patient Name: Nagwa Abd El Rahman Abo Ouf Abd El Rahman	Date : 09/10/2022
Exam : ultrasound BREAST (BOTH SIDE)	Age : 58 Year(s)
Medical ID: 2932/2021	Pat. ID: 21050

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Patient had right CBS

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B).
- No left breast suspicious mass lesions, micro calcifications or architectural distortion detected.
- Right breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed right breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions. Small operative bed seroma measures 0.6 x 1.8 cm.
- no left breast solid or cystic mass lesions, normal skin thickness and nipple-areola complex.
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Yours truly

Signature Dr. Aya Deabes

09/10/2022 4:31 PM

Patient name: Nagwa Abdel Rahman

Date: 10/7 /2021

Combined bilateral breast mammography and complementary ultrasound

Indication:

- = Right breast lump

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibroglandular densities (ACR B)
- Right mid inner deeply seated irregular shaped hyperdense mass lesion is seen associated with architectural distortion and suspected intra-ductal extension. By complementary ultrasound, it corresponds to 3 o'clock irregular shaped hypoechoic mass lesion measures about 2.4 x 1.6 cm with no ultrasound evidence of intra-ductal extension to the nipple/areola complex.
- Bilateral prominent retro-areolar ducts are note.
- Normal skin thickness.
- Normal nipple-areola complex of the left breast.
- Bilateral Nonspecific axillary LNs are noted showing within average cortical thickness and preserved central fatty hila.

Opinion:

- *Right 3 o'clock highly suspicious mass lesion (BIRADS 4c), for histopathological correlation.*
- *Further evaluation by contrast enhanced study is recommended for proper delineation of its extent.*

Yours truly,

Dr. Sherihan Fakhry, MD